



Provincial Health Services Authority

Air Ambulance and Clinical Air Operations

Flight Paramedic

Operational Scope of Practice Matrix for

Clinical Advisors & Dispatchers



Version 2.2

Current Amendments

Date	Version	Description
01.21.26	1.1	Previous amendments found at end of document
02.14.26	2.0	
03.05.26	2.1	

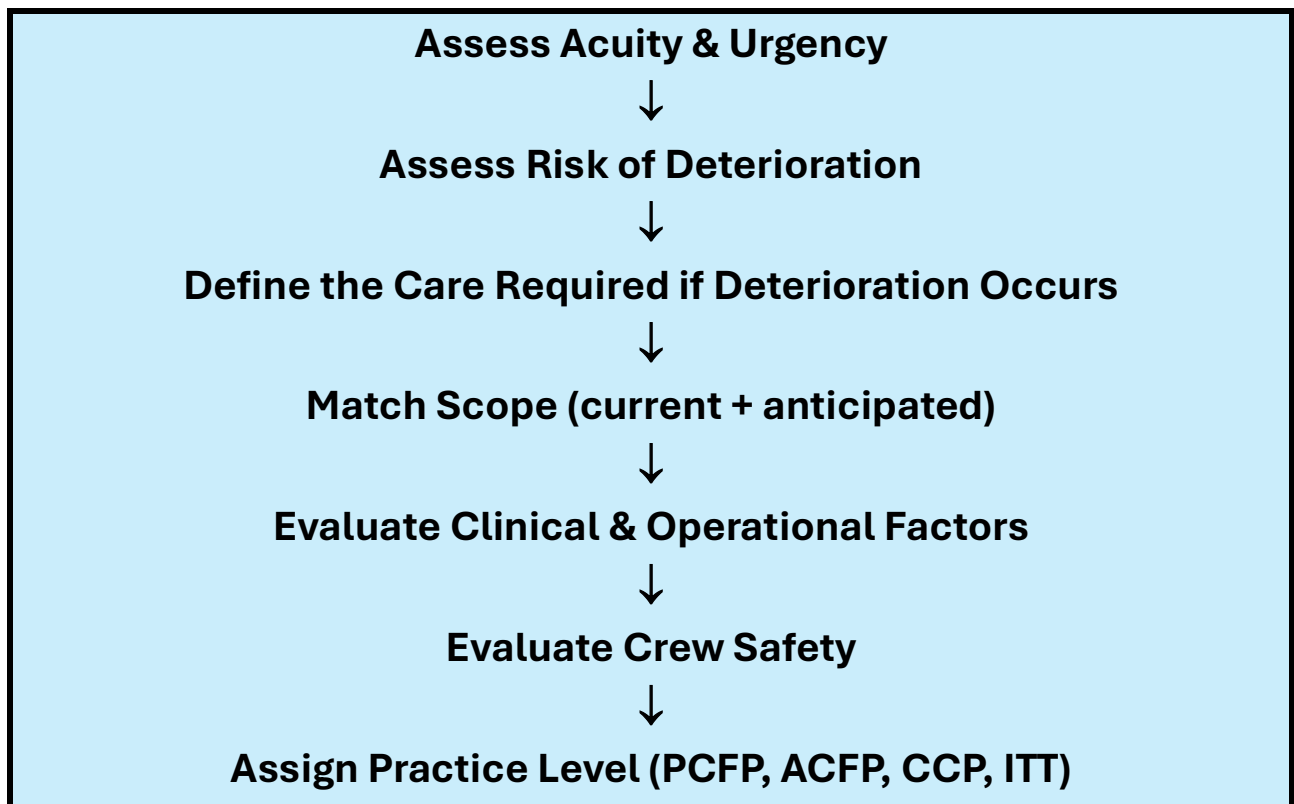
04.14.26	2.2	Description
Section 2		
4.0 Oxygenation and Ventilation: Invasive Mechanical Ventilation	Amended ACFP to Operational Restriction	

Definitions

The following definitions will be used and can be referenced to the Emergency Medical Assistants (EMA) Regulation

PCFP	Primary Care Flight Paramedic The category specified in the following EMA Regulation: Schedule 1 Section 3, Schedule 2 Sections 2, Schedule 2 Section 3.
ACFP	Advanced Care Flight Paramedic The category specified in the following EMA Regulation: Schedule 1 Section 4, Schedule 2 Sections 3, Schedule 2 Section 4.
ITT	Infant Transport Team. The category specified in the following EMA Regulation: Schedule 1, Section 6, Schedule 2, Section 3
CCP-MNP	Critical Care Paramedic – Maternity, Neonatal & Paediatric Specialisation. The category specified in the following EMA Regulation Schedule 1, Section 5, Schedule 2, Section 4.
CCP-Adult	Critical Care Paramedic – Adult Specialisation. The category specified in the following EMA Regulation Schedule 1, Section 5, Schedule 2, Section 4.

1. Clinical Advisor's Approach & Mission Profiles



To assist with decision-making when deploying a Clinical Air Operations asset, Clinical Advisors and Dispatchers may refer to the following mission profiles as defined in the BCEHS Air Ambulance and Critical Care Operations Standard Operating Procedures.

Primary Care Flight Paramedic (PCFP)

Primary Role

Provides select scene response and interfacility transport of various **adult and pediatric** patients within their scope of practice. These are **low complexity** patients who have been **differentiated, resuscitated**, and require **maintenance** of treatment during transport, and who have a **low** chance of deterioration.

Secondary Role

Time sensitive transport of a **high-acuity, low complexity** patient requiring logistically reasonable **immediate** transport that is within PCFP scope of practice.

Advanced Care Flight Paramedic (ACFP)

Primary Role

Provides select scene response and interfacility transport of acutely ill and injured **adult and pediatric** patients within their scope of practice. These are **low to moderate complexity** patients who have been **differentiated, resuscitated**, but require **some level of care** during transport, and have a **moderate** chance of deterioration.

Secondary Role

Time sensitive transport of a **high-acuity, low to moderate complexity** patient requiring logistically reasonable **immediate** transport that is within ACFP scope of practice.

Critical Care Paramedic – Adult (CCP-Adult)

Primary Role

Provides dedicated scene response, retrieval, and interfacility transport of **critically** ill and injured **adult** and **select pediatric** patients across the province. These are **complex** patients who may be **undifferentiated, under-resuscitated**, and have a **high-risk** of deterioration during transport.

Secondary Role

Events where **PCFP, ACFP, ITT or CCP-MNP** have **no-capacity to respond**. Transport must be **fiscally responsible, logistically reasonable** and within **CCP-Adult scope** of practice.

Infant Transport Team (ITT)

Primary Role

Provides select scene response, retrieval and **specialized** interfacility transport of **critically** ill and injured **neonatal, pediatric, and high-risk maternity** patients across the province requiring **ongoing care** during transport and who have a **high risk** of deterioration during transport.

Secondary Role

Time sensitive transport of a **high-acuity, low to moderate complexity** patient requiring logistically reasonable **immediate** transport that is within ITT scope of practice.

Critical Care Paramedic – MNP (CCP-MNP)

Primary Role

Provides dedicated scene response, retrieval, and interfacility transport of **critically** ill and injured **complex neonatal, pediatric, and high-risk maternity** patients across the province requiring **ongoing care** and who are at a **high-risk** for deterioration during transport.

Secondary Role

Events where **PCFP, ACFP, ITT or CCP-Adult** have **no-capacity to respond**. Transport must be **fiscally responsible, logistically reasonable** and within **CCP-MNP scope** of practice.

2. Skills Authorized for Use, by Clinical Role

This section presents clinical skills authorized to be practiced by the different clinical roles within the operational response framework of Air Ambulance and Clinical Air Operations

The following list indicates the major endorsed activities relevant to the paramedic operational scope of practice within BCEHS.

- | | |
|---|-------------------------------|
| 1. Response Capability | 5. Initiating Vascular Access |
| 2. Primary Assessment and Intervention | 6. Assessment and Monitoring |
| 3. Airway Management | 7. Procedures |
| 4. Oxygenation and Ventilation Administration | 8. Routes of Medication |

The following colour code identifiers are used within the matrix to indicate the baseline expectations and recommendations of operational practice for each flight paramedic level.

Advisors and dispatchers should note that amendments to the PCFP and ACFP operational scope of licensed activities will occur as training and field practice evolves.

Identifier	Definition
	Within BCEHS independent operational scope
CC	Within BCEHS independent operational scope with clinical consultation recommended
Mod	Modified BCEHS independent operational scope
A	Within BCEHS operational scope but requires mandatory clinical advisor consultation
	Not within legal scope for the licence level
OR	Operationally restricted by BCEHS

1.0 Response Capability

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation / Guidelines	Notes
Modes	Routine (Cold)						EHS Act	
	Emergency (Hot)							
Platform	Ground							
	Marine							
	Air (Rotary & F/W)							
Complexity	Low	CC	CC	CC	CC	CC	EMA Regulations Schedule 1 & 2	Patient acuity may require clinical consult
	Moderate	A	CC	CC	CC	CC	BCEHS Clinical Air Operations SOPs	
	Complex	A	A	CC	CC	CC		
Transport	Lifting & loading Systems						Schedule 1 Section 1(l)	
	Incubator for thermoregulation					OR	Schedule 1 Section 5(c) Schedule 1 Section 6(k)	
	Incubator transport platform non-thermoregulation	CC	CC				As per EMALB decision letter February 27 th 2026	
Inter-agency Events	Multidisciplinary scenes & sites						EMA Regulations Schedule 3	
Response Safety	Helicopter underwater egress						Transport Canada Canadian Aviation Regulations 401.05 (2)(a) Transport Canada Canadian Aviation Regulations 602.66(1)(c)	Pro-Aviation Safety Training
	Use of Carbon Monoxide detection device						Schedule 2 Section 1(i)	
ECLS	Transport	A	A	A	A	A	Schedule 1 Section 1(l)	PCFP & ACFP may transport ECMO Team to Sending facility

2.0 Primary Assessment

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation / Guidelines	Notes
Scene	Assessment & Risk Analysis						Schedule 1 Section 1(a)	
Primary	Basic Assessment & Interventions						Schedule 1 Section 1(b)	See section 5, 6, 7 & 8 for specific assessment and interventions
Secondary	History, Vitals, Head-to-toe						Schedule 1 Section 1(c) Schedule 2 Section 2(e)	
Recognition of Life-extinct	Confirm evidence for non-resuscitation or discontinuation	CC	CC	CC	CC	CC	Schedule 1 Section 1(b)(c)	Discontinuation of resuscitation orders may be required
Critical Cardiac Intervention	High-performance CPR						Schedule 1 Section 1(d)	
	Mechanical CPR	OR		A	A			Patient size restrictions apply
	Automated defibrillation						Schedule 1 Section 1(i)	
	Manual defibrillation						Schedule 2 Section 3(j)	
	Synchronised cardioversion						Schedule 1 Section 4(b) Schedule 1 Section 6(m)	
	Transcutaneous pacing							

3.0 Airway Management

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Airway Adjuncts	Nasopharyngeal airway insertion						Schedule 1 Section 1(g)(i)	See section 5, 6, 7 & 8 for specific assessment and interventions
	Oropharyngeal airway insertion						Schedule 1 Section 1(g)(i)	
	Supraglottic airway insertion						Schedule 1 Section 3(c) Schedule 1 Section 4(j)	
	Endotracheal Intubation	OR					Schedule 2 Section 3(c)	PCFP operational restriction
Airway Access	Direct laryngoscopy	OR					Schedule 1 Section 1(g)(i)	
	Video laryngoscopy	OR						
	Use of Magill forceps	OR						
	Cricothyrotomy						Schedule 1 Section 4(g)	
Airway Suctioning	Oral suctioning						Schedule 1 Section 1(g)(ii)	
	Supraglottic airway device suctioning						Schedule 1 Section 3(c) Schedule 1 Section 4(j)	
	Endotracheal tube suctioning	Mod					Schedule 1 Section 1(g)(ii)	PCFP may not suction beyond the length of the tube
	Tracheostomy tube suctioning	Mod						

4.0 Oxygenation and Ventilation

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Oxygenation	Oxygen administration						Schedule 1 Section 1(h)	
Non-Mechanical Ventilation	Bag-valve-mask						Schedule 1 Section 1(g)(ii)	
	Non-invasive CPAP						Schedule 2 Section 3(e)	
	Non-invasive PEEP							
Non-invasive Mechanical Ventilation	CPAP						Schedule 2 Section 4(c) Schedule 1 Section 6(f)	BiPAP via a mechanical ventilator is not sufficiently automated and requires some patient management versus maintenance only.
	BiPAP		OR					
	High Flow							
Invasive Mechanical Ventilation	Automated		OR				Schedule 2 Section 4(a)	Requires EMALB endorsement
	Strategy development and implementation			CC	CC	CC	Schedule 1 Section 5(i) Schedule 1 Section 6(f)	Requires ventilator management
	Recruitment manoeuvres			CC	CC	CC		Consultation on ventilator strategy required if different from sending facility care
	Prone position ventilation			CC	CC	CC		

5.0 Initiating Vascular Access

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Peripheral Vascular Access	Initiate peripheral venous access						Schedule 1 Section 3(e)	
	Initiate external jugular venous access						Schedule 1 Section 4(f)	
	Initiate umbilical venous access					OR	Schedule 1 Section 6(u)	
	Arterial access for sample collection						Schedule 1 Section 6(p) Schedule 2 Section 4(g)	
	Arterial line insertion						Schedule 1 Section 5(b)	Restricted for patients less than 12 yrs.
	Obtain capillary blood sample						Schedule 1 Section 3(f) Schedule 2 Section 3(g)	
	Obtain venous blood sample							
	Obtain arterial sample via VAMP						Schedule 1 Section 6(p) Schedule 2 Section 4(g)	
Parenteral Feeding	Peripheral insertion						Schedule 1 Section 6(r)	ACFP requires consult All access for PNN only.
Central Venous Access	Central line insertion				OR	OR	Schedule 1 Section 5(e)	

6.0 Assessment and Monitoring

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
General Assessment	Chest auscultation						Schedule 2 Section 2(e)	
	Manual blood pressure						Schedule 2 Section 2(n)	
	Non-invasive blood pressure							
	Pulse oximetry						Schedule 2 Section 1(i)	
	Capnography						Schedule 1 Section 3(g)	
	Glucometer						Schedule 2 Section 1(j)	
	Non-invasive temperature						Schedule 1 Section 1(b)	
Lab Data	Lab data interpretation						Schedule 2 Section 4(h) Schedule 1 Section 6(q)	
	ABG interpretation							
	Urinalysis							
Radiological Data	Basic CXR/CT/MRI interpretation							
Point-of-Care Ultrasound	Acquisition and interpretation	Mod	Mod				Schedule 1 Section 3(f)	PCFP and ACFP: Ultrasound-guided peripheral intravenous catheter placement only
ECG	3-lead interpretation and monitoring						Schedule 2 Section 3(k)	
	12-lead acquisition						Schedule 2 Section 3(d)	
	12-lead interpretation	OR					Schedule 2 Section 3(j)(k)	PCFP restricted to 3-lead interpretation only
EEG	Acquisition, interpretation and monitoring	OR	OR				Schedule 1 Section 3(f)	EEG: PCFP/ACFP operational restriction

6.1 Assessment and Monitoring

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Invasive Monitoring	Arterial line monitoring						Schedule 2 Section 4(d) Schedule 1 Section 6(g)	
	Central venous line monitoring							
	Oesophageal manometry						Schedule 1 Section 5(g)	
	ICP monitoring							
	Oesophageal temperature						Schedule 1 Section 1(b)	
	Rectal temperature							
	Abdominal compartment pressure monitoring							
	Bladder pressure evaluation							

7.0 Procedures

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
General Anaesthesia	Anaesthesia planning						Schedule 1 Section 4(m)	
	Sedation-facilitated intubation	OR					Schedule 2 Section 3(c)	
	Rapid sequence intubation	OR					Schedule 2 Section 3(c)	
Local Anaesthesia	Subcutaneous anaesthetic						Schedule 2 Section 4(o) Schedule 1 Section 6(s)	
	Intraosseous anaesthetic						Schedule 2 Section 3(h)	
Cardiac	Transvenous pacing		Mod				Schedule 2 Section 4(m)	ACFP may transport only for stable patients and as maintenance only.
	Pericardiocentesis				OR	OR	Schedule 1 Section 5(h)	
Chest Decompression	Needle thoracentesis						Schedule 2 Section 3(l)	
	Finger thoracostomy		OR	OR	OR	OR	Schedule 2 Section 4(p)	
	Chest tube insertion				OR	OR	Schedule 1 Section 5(d)	
	Pigtail catheter thoracostomy					OR	Schedule 1 Section 5(d) Schedule 1 Section 6(t)	
	Chest tube maintenance	A						PCFP: Chest tube not requiring continuous suction or management
	Chest tube management		A				Schedule 2 Section 4(j) Schedule 1 Section 6(j)	Advisor must evaluate if safe to transport within current scope of practice and consider potential for risk of deterioration during transport
	Chest underwater drainage device	A	A				Schedule 2 Section 4(j) Schedule 1 Section 6(j)	
	Chest tube securement with sutures						Schedule 2 Section 4(n) Schedule 1 Section 6(i)	

7.1 Procedures

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Gastro-intestinal	Orogastric tube insertion						Schedule 1 Section 4(h) Schedule 1 Section 6(h)	
	Nasogastric tube insertion							
	Gastric tube suctioning							
	Oesophageal manometer insertion						Schedule 1 Section 5(g)	
	Abdominal drains (wicking)							
	Abdominal drains (negative pressure)		A				Schedule 2 Section 4(n)	ACFP: Must be evaluated by advisor to determine if safe to transport
Genito-urinary	Urinary catheter insertion		CC				Schedule 1 Section 6(o) Schedule 2 Section 4(c)	
Infectious Disease	Microbiology swabbing						Schedule 2 Section 3(i)	
Maternity	Emergency childbirth						Schedule 2 Section 1(f)	
Musculo-skeletal injury	Fracture management						Schedule 1 Section 1(k)	
	Joint dislocation reduction		CC				Schedule 1 Section 4(n)	Neuromuscular or vascular compromise must exist
	Pelvic binding						Schedule 1 Section 1(k)	
	C-spine clearance							
	Spinal motion restriction (SMR)						Schedule 2 Section 1(c)	
	Spinal immobilisation (C-collar)						Schedule 1 Section 1(k)	

7.2 Procedures

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Patient Restraint	Physical restraint						Schedule 2 Section 3(n)	Least Restraint Policy Clinical Practice Guidelines: G01;G01.1: Behavioural Emergencies G03;G03.1: IFT of Behavioural Compromised Patients. Pre-transport Risk Assessment Tool
	Mechanical restraint							
	Pharmacological restraint	A						
Wound Management (Non-invasive)	Wound examination						Schedule 1 Section 1(j) Schedule 1 Section 2(j)	
	Wound cleaning							
	Non-invasive wound closure							
	Dressing and bandaging							
	Vascular compression bandage							
	Tourniquet							
	Chest seal							
Wound Management (Invasive)	Wound closure (Suturing)						Schedule 2 Section 4(n)	
	Wound closure (Glue)							
	Wound closure (Staples)							
	Wound packing (Non-medicated)						Schedule 1 Section 2(j)	
	Nasal packing	CC					Schedule 1 Section 1(j)	
	Wound packing (Hemostatic)						Schedule 2 Section 1(k)	
	Escharotomy						Schedule 2 Section 5(f)	

8.0 Routes of Medication Administration

Practice Area	Procedure	PCFP	ACFP	ITT	CCP-MNP	CCP-Adult	Act / Regulation Guidelines	Notes
Non-vascular Routes	Oral						Schedule 1 Section 2(a)	
	Sublingual							
	Intranasal							
	Inhaled/nebulised						Schedule 1 Section 2(c)(h)	
	Rectal						Schedule 1 Section 4(m)	
	Topical						Schedule 2 Section 1(l)	
	Subcutaneous						Schedule 1 Section 3(b)	
	Intramuscular						Schedule 2 Section 2(g)	
Vascular Routes	Intravenous							All scope levels may administer medication infusions for purposes of maintenance Maintenance administration may require consultation with clinical advisor for some routes and medications. Where indicated, clinical consult is mandatory.
	External jugular	CC						
	Intraosseous	CC						
	Umbilical venous line	A	A			A	Schedule 1 Section 6(u) Schedule 1 Section 5(e)	
	Parenteral feed line	A	A			A	Schedule 2 Section 4(l) Schedule 1 Section 6(r)	
	Central venous line	A	A				Schedule 1 Section 3(d) Schedule 2 Section 3(n)	

3. Authorised Medications

This section represents the authorised medications that may be administered by Flight Paramedics during operational practice.

The matrix follows a similar colour coding as the previous and is further classified by a level of risk of medications being used.

Identifier	Definition
	Within BCEHS independent operational scope
CC	Within BCEHS independent operational scope with clinical consultation recommended
A	Within BCEHS IFT operational scope but requires mandatory clinical advisor consultation
R	Restricted BCEHS IFT operational scope requiring risk stratification and mission planning with clinical advisory group
	Medications anticipated to be used within the MNP Transport setting
	Standard formulary for Clinical Air Operations kit

Clinical advisors should anticipate that the maintenance of a medication during transport may include continuous intravenous infusions or the scheduled administration of a medication via a different route. The use of medication that requires the paramedic to adjust the dose in response to changes in the patient's condition is considered management and may be outside of IFT operational scope for a given level of practice.

Within BCEHS, medications outside of standard legal scope are not typically recommended for patient management during an IFT, however, medications that fall outside of the green and orange categories may still be administered for maintaining the care plan initiated by the sending facility under **EMALB Regulation 2.3(n)(i)(ii)**. **Requests for the modification of a predetermined Individualised Transport Plan developed under this regulation require EPOS or TA approval.**

Under circumstances where secondary mission needs are a priority to ensure LLTO transport is carried out, minimal forms of management can be considered following appropriate risk stratification and consultation on the Individualised Transport Plan.

Advisors should consider the risk of change/deterioration of the patient's condition, especially the impact the transport environment may have on oxygen demand, hemodynamics, sedation and pain control before supporting the Individualised Transport Plan. If in the clinical judgement of the advisor there is a concern with the alignment of the transporting paramedic's scope of practice and the patient's anticipated clinical needs during transport, a re-conferencing should take place to address the transport concerns and if required, an alternate crew with appropriate scope of practice should be assigned to the transport instead.

To help assess potential risks of medication administration, common medications have been categorised by level of risk. These are recommendations and depending on the circumstances, the clinical advisor can determine best practice versus minimally acceptable practice in any given situation.

High risk medications

Defined for this purpose as bearing a heightened risk of causing significant harm when used incorrectly.

- This includes medications on the [high alert drugs list](#) from ISMP and would include infusions where titration is required:
 - Anesthetics
 - Anti-thrombotics
 - Chemo
 - Electrolytes
 - Insulin
 - Neuromuscular blockers
 - Opioids and benzodiazepines
 - Medications with profound cardiovascular/physiologic effects
- CTS medications are listed in **RED TYPE**
- To use these medications
 - 1) it should be operationally approved within BCEHS which is indicated by inclusion in the BCEHS Handbook Drug Monographs and may include additional education.
 - 2) EPOS should be contacted for approval for anything that requires a medical advisor consult.
 - 3) If any high risk medications are not considered operationally approved but are deemed reasonable by the transport advisor team given the circumstance, please make a decision that supports best patient care. Clearly document the rationale for use in the ePCR and follow up with Clinical Advisory Team.

High Risk					
Medication	PCFP	ACFP	ITT	CCP	Notes
N-Acetylcysteine	A	A	CC	CC	2.1(m)
Adenosine	R				1.4(m)(i) / 1.6(i)
Albumin	R	A			2.4(e)
Amiodarone	A	CC	CC	CC	1.4(m)(i) / 1.6(m)
Atropine	A				2.3(f)(v)
Calcium Chloride	R				1.4(m)(ii)
Calcium Gluconate	R				1.4(m)(ii)
Cisatracurium	R	A	CC	CC	1.4(o) / 1.6(l)
Cryoprecipitate	R	A	CC	CC	1.4(o) / 1.6(l)
Dexmedetomidine	R	A	CC	CC	1.4(m)(viii)
Diazepam	R	A	CC	CC	1.4(m)(viii)
Diltiazem (iv)	A	A	CC	CC	1.4(m)(xi)
Dobutamine	R	R	CC	CC	1.3(b)(iv)
Dopamine	R	A	CC	CC	1.3(b)(iv)
Enoxaparin	R	A	CC	CC	1.4(m)(iv) / 1.6(l)
Epinephrine					1.3(b)(iv)
Eptifibatide	R	R	CC	CC	1.4(m)(iv) / 1.6(l)
Etomidate	R	R	A	A	1.4(o) / 1.6(l)
Fentanyl	CC				2.3(n)(i)(ii)
Heparin (unfractionated)	A	A	CC	CC	2.3(n)(i)(ii) / 1.4(m)(iv)
Hydromorphone	CC	CC	CC	CC	2.3(f)(iv)
Hypertonic Saline 3%	R	A	CC	CC	1.4(o) / 1.6(l)
Immune Globulin	R	A	CC	CC	2.4l / 1.6(j)
Insulin	R	A			1.4(0) / 2.3(n)(i)(ii)
Isoproterenol	A	A	CC	CC	1.3(b)(iv)
Ketamine	CC				1.3(b)(viii)
Labetolol (iv)	R	A	CC	CC	1.4(m)(i) / 1.6(d)(i)
Lidocaine					1.4(m)(i)/1.6(d)(i)/2.3(h)
Lorazepam	A	CC	CC	CC	1.4(m)(viii) / 1.6(d)(vi)
Mannitol	A	A			1.4(m)(iii) / 2.3(n)(i)(ii)
Metoprolol (iv)	R				1.4(m)(i) / 1.6(d)(i)
Midazolam	R				1.4(m)(viii)
Milrinone	R	A			2.3(n)(i)(ii)
Morphine Sulphate	CC				1.4(o) / 1.6(l)
Nitroglycerin (infusion)	A	A			2.3(n)(i)(ii)
Nitric Oxide	R	R	CC	CC	1.6(l) /
Norepinephrine	R	A	CC	CC	1.3(b)(iv)
Packed Red Blood Cells (PRBC)	R	A			2.4(e) / 1.6(j)
Phenylephrine	A				1.3(b)(iv)
Phenobarbital	R	CC			1.6(d)(ix)
Phenytoin	A	A			2.3(n)(i)(ii)
Potassium Chloride	R	A			1.4(o)
Procainamide	R	A			1.4(m)(i)
Propofol	R	A			1.4(m)(viii)
Prothrombin Complex	R	A			1.4(e) / 1.6(j)

Rocuronium	R	A			1.4(o)
Succinylcholine	R	A			1.4(o)
Tenectaplast	R	A			1.4(o) / 2.3(n)(i)(ii)
Vasopressin	A	A			2.3(n)(i)(ii)

Moderate Risk Medications

Moderate risk medications are those that are deemed to cause moderate risk of harm when used incorrectly. Of note, some high-risk medications have been deemed moderate risk when used as single doses versus infusions.

- The standard formulary is highlighted in blue. For medications that are outside of that, ideally first doses should have been administered prior to initiating transfer or transfusions should be initiated for a minimum of one hour prior to sending.
- Pink highlighting indicates medications anticipated to be used within the PCFP/ACFP-MNP transport environment.
- Where inclusion in the BCEHS Handbook and formulary does not exist, using the medications from the sending site along with a copy of instructions for their parenteral manual is reasonable.
- If you are unable to obtain a copy of the local monograph use the [Vancouver Coastal Parenteral Monograph](#)
- EPOS consultation is required for any medication requiring a medical advisor consult.

Moderate Risk					
Medication	PCFP	ACFP	ITT	CCP	Notes
Ampicillin (IV)	A	A	CC	CC	2.3(n)(ii)(iii) / 1.6(l)
Desmopressin	R	A	CC	CC	1.4(o) / 1.6(l)
Dextrose (IV)					1.3(b)(vi)
Fomepizole	A	A	CC	CC	2.3(n)(i)(ii)
Fosphenytoin	A	A	CC	CC	2.3(n)(i)(ii)
Furosemide	A	A	CC	CC	1.4(m)(iii) / 2.3(n)(i)(ii)
Gentamicin	A	A			2.3(n)(i)(ii)
Glucagon					1.3(b)(vi) / 1.6(d)(v)
Haloperidol	A	A	A	CC	2.3(n)(i)(ii)
Hydralazine	A	A			2.3(n)(i)(ii)
Hydrocortisone					2.3(f)(iii)
Hydroxocobalamin					2.3(f)(ii)
Ketorolac					1.3(b)(viii)
Labetolol (PO)	R	A	CC	CC	1.4(m)(i) / 1.6(d)(i)
Levetiracetam*	R	A			1.4(m)(xi) / 1.6(d)(ix)
Magnesium Sulphate	A				1.4(m)(xi) / 1.6(d)(ix)
Mannitol	A	A			1.4(m)(iii) / 2.3(n)(i)(ii)
Metoclopramide	A	A			1.4(m)(xi) / 1.6(d)(vii) / 2.3(f)(i)

Methylprednisolone	A	A			2.3(f)(iii)
Nifedipine	R	A	CC	CC	2.2(c)(i)
Nimodipine	A	A	R	CC	2.3(n)(i)(ii)
Nitroglycerin SL, oral					2.2(c)(i)
Octreotide	A	A			2.3(n)(i)(ii)
Ondansetron (IV)					2.3(f)(i)
Oxytocin	A	A			1.6(l)
Pantoprazole (IV)	A	A			2.3(n)(i)(ii)
Penicillin G	A	A			2.3(n)(i)(ii)
Protamine Sulphate	R	A			1.4(o)
Sodium Chloride 0.9%					1.3(d)
Tranexamic Acid					1.3(b)(v)
Vitamin K					2.3(f)(ii)

Low Risk Medications

Low risk medications are those that are deemed to cause moderate risk of harm when used incorrectly.

- These medications can be administered at any time during the flight and information should be guided by the BCEHS Handbook or tertiary resources such as UpToDate.
- EPOS consultation is required for any medication requiring a medical advisor consult.

Low Risk					
Medication	PCFP	ACFP	ITT	CCP	Notes
Acetaminophen (tablet, liquid, IV)					2.2(c)(iii) / 1.6(iii)
Acetylsalicylic Acid					2.1(q) / 2.2(iii)
Clopidogrel	A	A			2.2(c)(iv)
Dextrose (PO)					1.3(b)(vi)
Dexamethasone (D10W)					2.3(f)(iii)
Dimenhydrinate					1.4(m)(ix) / 2.3(f)(i)
Diphenhydramine					1.3(b)(iii) / 1.4(m)(x)
Gluco Gel					1.3(b)(vi) / 1.6(d)(v)
Ibuprofen					2.2(c)(iii)
Ipratropium Bromide (MDI)					2.3(f)(v)
Naloxone					1.3(b)(i)
Nitrous Oxide					2.2(c)(iii)
Ondansetron (PO)					2.3(f)(i)
Oxygen					1.1(h)
Pantoprazole (PO)	A	A			2.3(n)(i)(ii)
Prednisone	A	A			2.3(f)(iii)
Salbutamol (MDI)					1.3(b)(iv)
Ticagrelor	A	A			2.3(n)(i)(ii)

4. Emergency Medical Assistants Regulations

Schedules 1 and 2 are provided for reference purposes and have been compiled to reflect each Flight Paramedic level of practice within their legal scope.

Sections that have been repealed have been removed and only the current legal scope is documented here.

**Legal Scope of Practice
Primary Care Flight Paramedic (PCFP)**

(Schedule 1.1)

- (a) scene assessment;
- (b) assessment of level of consciousness, skin colour and temperature, pulse, and respiration;
- (c) rapid body survey to identify and attend to any life threatening injuries followed by a secondary assessment consisting of a physical examination, medical and incident history, and vital signs;
- (d) cardiopulmonary resuscitation;
- (g) maintenance of airways and ventilation, including by
 - (i) insertion and maintenance of oropharyngeal airway devices and nasopharyngeal airway devices, and
 - (ii) use of suction devices and bag-valve-mask devices;
- (h) oxygen administration and use of oxygen administration equipment;
- (i) use of automatic and semi-automatic defibrillators;
- (j) wound management not requiring tissue puncture or indentation;
- (k) fracture management and immobilization;
- (l) lifting/loading and extrication/evacuation.

(Schedule 1.2)

- (b) occupational first aid;
- (c) transportation;
- (j) soft tissue injury treatment.

(Schedule 1.3)

- (b) administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i) opioid antagonist;
 - (iii) anti-histaminic;
 - (iv) sympathomimetic agent;
 - (v) procoagulant;
 - (vi) anti-hypoglycemic agent;
 - (vii) anti-pyretic;
 - (viii) non-opioid analgesics;
- (c) insertion and maintenance of airway devices not requiring visualization of the larynx;
- (d) maintenance of intravenous lines, including use of infusion devices and saline locks;
- (e) initiation of peripheral intravenous lines;
- (f) point-of-care testing to inform emergency treatment or transportation decisions or to provide information for subsequent treatment;
- (g) end-tidal carbon dioxide monitoring.

PCFP Endorsements

(Schedule 2.1)

- (c)spinal motion restriction;
- (e)administration of oral glucose;
- (f)emergency childbirth;
- (h)assistance to a patient with the administration of a medication to the patient, provided that
 - (i)if, under the *Pharmacy Operations and Drug Scheduling Act*, a particular medication is available only on the prescription of a health professional, the medication has already been prescribed to the patient by a health professional,
 - (ii)the patient has requested the EMA to assist the patient with the administration of the medication,
 - (iii)the administration of the medication is related to emergency services the EMA is providing to the patient,
 - (iv)the medication is administered as prescribed, and
 - (v)the patient is not being transported between health facilities;
- (i)use and interpretation of pulse oximeters and CO-oximeters;
- (j)use and interpretation of glucometers;
- (k)administration of epinephrine by intramuscular auto-injector or intranasal or sublingual preparations;
- (l)topical administration of pro-coagulants and anti-fibrinolytics;
- (m)intramuscular and intranasal administration of opioid antagonists;
- (n)non-invasive blood pressure measurement;
- (o)intramuscular and intranasal administration of anti-hypoglycemic agents;
- (p)administration of oral analgesics;
- (q)administration of acetylsalicylic acid

(Schedule 2.2)

- (a)maintenance of intravenous lines without medications or blood products;
- (c)administration of the following oral, sublingual or inhaled medications:
 - (i)anti-anginal;
 - (iii)analgesics;
 - (iv)platelet inhibitors;
- (e)chest auscultation;
- (g)intramuscular administration of epinephrine;
- (h)administration of bronchodilators by inhalation and nebulization.

(Schedule 2.3)

- (b)administration of isotonic crystalloid solutions;
- (c)endotracheal intubation;
- (d)electrocardiogram acquisition;
- (e)initiation and maintenance of non-invasive positive pressure airway devices;
- (f)administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i)anti-emetic — anti-nauseant;
 - (ii)vitamins;

- (iii)corticosteroids;
- (iv)opioid analgesics;
- (v)anti-cholinergic;
- (g)collection of capillary and venous blood samples;
- (h)initiation and maintenance of intraosseous needle cannulation with local anesthetic instillation;
- (i)taking microbiology swabs of dermal and mucosal sites;
- (j>manual defibrillation with cardiac rhythm interpretation;
- (k)electrocardiogram interpretation;
- (l)needle thoracentesis;
- (m)administration of chemical and biological agent countermeasures;
- (n)administration of drug therapy during transportation between facilities
 - (i)in accordance with a treatment plan issued by the patient's attending medical practitioner at the facility from which the patient is being transported, and
 - (ii)after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

**Legal Scope of Practice
Advanced Care Paramedic (ACFP)**

(Schedule 1.1)

- (a) scene assessment;
- (b) assessment of level of consciousness, skin colour and temperature, pulse, and respiration;
- (c) rapid body survey to identify and attend to any life threatening injuries followed by a secondary assessment consisting of a physical examination, medical and incident history, and vital signs;
- (d) cardiopulmonary resuscitation;
- (g) maintenance of airways and ventilation, including by
 - (i) insertion and maintenance of oropharyngeal airway devices and nasopharyngeal airway devices, and
 - (ii) use of suction devices and bag-valve-mask devices;
- (h) oxygen administration and use of oxygen administration equipment;
- (i) use of automatic and semi-automatic defibrillators;
- (j) wound management not requiring tissue puncture or indentation;
- (k) fracture management and immobilization;
- (l) lifting/loading and extrication/evacuation.

(Schedule 1.2)

- (b) occupational first aid;
- (c) transportation;
- (j) soft tissue injury treatment.

(Schedule 1.3)

- (b) administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i) opioid antagonist;
 - (iii) anti-histaminic;
 - (iv) sympathomimetic agent;
 - (v) procoagulant;
 - (vi) anti-hypoglycemic agent;
 - (vii) anti-pyretic;
 - (viii) non-opioid analgesics;
- (c) insertion and maintenance of airway devices not requiring visualization of the larynx;
- (d) maintenance of intravenous lines, including use of infusion devices and saline locks;
- (e) initiation of peripheral intravenous lines;
- (f) point-of-care testing to inform emergency treatment or transportation decisions or to provide information for subsequent treatment;
- (g) end-tidal carbon dioxide monitoring.

(Schedule 1.4)

- (b) cardioversion and external pacing;
- (f) initiation of external jugular vein cannulation;
- (g) cricothyrotomy;
- (h) gastric intubation and suction;

- (i)maintenance of intravenous lines with medications;
- (j)insertion and maintenance of advanced airway devices which do not require laryngoscopy;
- (l)administration of colloid and non-crystalloid volume expanders;
- (m)administration of the following intravenous, oral, nebulized, endotracheal, intraosseous, intramuscular and rectal medications:
 - (i)anti-arrhythmic;
 - (ii)electrolyte — calcium therapy;
 - (iii)diuretic;
 - (iv)anti-coagulant;
 - (viii)sedative;
 - (ix)anti-emetic — anti-nauseant;
 - (x)histamine antagonist;
 - (xi)anti-convulsant;
 - (xii)alkalizer;
- (n)reduction of joint dislocations involving neurological or vascular compromise;
- (o)administration of drug therapy after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

ACFP Endorsements

(Schedule 2.1)

- (c)spinal motion restriction;
- (e)administration of oral glucose;
- (f)emergency childbirth;
- (h)assistance to a patient with the administration of a medication to the patient, provided that
 - (i)if, under the [*Pharmacy Operations and Drug Scheduling Act*](#), a particular medication is available only on the prescription of a health professional, the medication has already been prescribed to the patient by a health professional,
 - (ii)the patient has requested the EMA to assist the patient with the administration of the medication,
 - (iii)the administration of the medication is related to emergency services the EMA is providing to the patient,
 - (iv)the medication is administered as prescribed, and
 - (v)the patient is not being transported between health facilities;
- (i)use and interpretation of pulse oximeters and CO-oximeters;
- (j)use and interpretation of glucometers;
- (k)administration of epinephrine by intramuscular auto-injector or intranasal or sublingual preparations;
- (l)topical administration of pro-coagulants and anti-fibrinolytics;
- (m)intramuscular and intranasal administration of opioid antagonists;
- (n)non-invasive blood pressure measurement;
- (o)intramuscular and intranasal administration of anti-hypoglycemic agents;
- (p)administration of oral analgesics;
- (q)administration of acetylsalicylic acid

(Schedule 2.2)

- (a)maintenance of intravenous lines without medications or blood products;
- (c)administration of the following oral, sublingual or inhaled medications:
 - (i)anti-anginal;
 - (iii)analgesics;
 - (iv)platelet inhibitors;
- (e)chest auscultation;
- (g)intramuscular administration of epinephrine;
- (h)administration of bronchodilators by inhalation and nebulization.

(Schedule 2.3)

- (b)administration of isotonic crystalloid solutions;
- (c)endotracheal intubation;
- (d)electrocardiogram acquisition;
- (e)initiation and maintenance of non-invasive positive pressure airway devices;
- (f)administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i)anti-emetic — anti-nauseant;
 - (ii)vitamins;
 - (iii)corticosteroids;
 - (iv)opioid analgesics;
 - (v)anti-cholinergic;
- (g)collection of capillary and venous blood samples;
- (h)initiation and maintenance of intraosseous needle cannulation with local anesthetic instillation;
- (i)taking microbiology swabs of dermal and mucosal sites;
- (j>manual defibrillation with cardiac rhythm interpretation;
- (k)electrocardiogram interpretation;
- (l)needle thoracentesis;
- (m)administration of chemical and biological agent countermeasures;
- (n)administration of drug therapy during transportation between facilities
 - (i)in accordance with a treatment plan issued by the patient's attending medical practitioner at the facility from which the patient is being transported, and
 - (ii)after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

(Schedule 2.4)

- (a)automated mechanical ventilation;
- (c)urinary catheterization;
- (d)arterial line management and central venous pressure monitoring;
- (e)infusion of blood products;
- (g)collection of arterial blood samples;
- (h)interpreting laboratory and radiologic data;
- (j)chest tube management;
- (k)central line management;
- (l)management of parenteral feeding lines and equipment;

- (m)provision of trans-venous pacing;
- (n)invasive wound management;
- (o)subcutaneous application of a local anaesthetic;
- (p)finger thoracostomy.

**Legal Scope of Practice
Infant Transport Team (ITT)**

(Schedule 1.1)

- (a) scene assessment;
- (b) assessment of level of consciousness, skin colour and temperature, pulse, and respiration;
- (c) rapid body survey to identify and attend to any life threatening injuries followed by a secondary assessment consisting of a physical examination, medical and incident history, and vital signs;
- (d) cardiopulmonary resuscitation;
- (g) maintenance of airways and ventilation, including by
 - (i) insertion and maintenance of oropharyngeal airway devices and nasopharyngeal airway devices, and
 - (ii) use of suction devices and bag-valve-mask devices;
- (h) oxygen administration and use of oxygen administration equipment;
- (i) use of automatic and semi-automatic defibrillators;
- (j) wound management not requiring tissue puncture or indentation;
- (k) fracture management and immobilization;
- (l) lifting/loading and extrication/evacuation.

(Schedule 1.2)

- (b) occupational first aid;
- (c) transportation;
- (j) soft tissue injury treatment.

(Schedule 1.3)

- (b) administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i) opioid antagonist;
 - (iii) anti-histaminic;
 - (iv) sympathomimetic agent;
 - (v) procoagulant;
 - (vi) anti-hypoglycemic agent;
 - (vii) anti-pyretic;
 - (viii) non-opioid analgesics;
- (c) insertion and maintenance of airway devices not requiring visualization of the larynx;
- (d) maintenance of intravenous lines, including use of infusion devices and saline locks;
- (e) initiation of peripheral intravenous lines;
- (f) point-of-care testing to inform emergency treatment or transportation decisions or to provide information for subsequent treatment;
- (g) end-tidal carbon dioxide monitoring.

(Schedule 1.6)

(d)administration of the following intravenous, oral, nebulized, endotracheal, intraosseous, intramuscular and rectal medications:

- (i)anti-arrhythmic;
- (ii)bronchodilator;
- (iii)anti-pyretic;
- (v)anti-hypoglycemic agent;
- (vi)sedative — anti-epileptic;
- (vii)anti-emetic — anti-nauseant;
- (viii)histamine antagonist;
- (ix)anti-convulsant;
- (x)alkalizer;

(f)mechanical ventilation;

(g)maintenance and monitoring of arterial and central venous catheters;

(h)gastric intubation and suction;

(i)management of chest tubes and chest drainage systems;

(j)intravenous blood product administration;

(k)use of incubators for thermoregulation;

(l)administration of drug therapy after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy;

(m)cardioversion and external pacing;

(n)cricothyrotomy;

(o)urinary catheterization;

(p)collection of arterial blood samples;

(q)interpreting laboratory and radiologic data;

(r)management of parenteral feeding lines and equipment;

(s)subcutaneous application of a local anesthetic;

(t)pigtail catheter placement for pneumothorax;

(u)umbilical vein catheterization and maintenance of venous access.

ITT Endorsements**(Schedule 2.1)**

(c)spinal motion restriction;

(e)administration of oral glucose;

(f)emergency childbirth;

(h)assistance to a patient with the administration of a medication to the patient, provided that

(i)if, under the [*Pharmacy Operations and Drug Scheduling Act*](#), a particular medication is available only on the prescription of a health professional, the medication has already been prescribed to the patient by a health professional,

(ii)the patient has requested the EMA to assist the patient with the administration of the medication,

(iii)the administration of the medication is related to emergency services the EMA is providing to the patient,

(iv)the medication is administered as prescribed, and

- (v)the patient is not being transported between health facilities;
- (i)use and interpretation of pulse oximeters and CO-oximeters;
- (j)use and interpretation of glucometers;
- (k)administration of epinephrine by intramuscular auto-injector or intranasal or sublingual preparations;
- (l)topical administration of pro-coagulants and anti-fibrinolytics;
- (m)intramuscular and intranasal administration of opioid antagonists;
- (n)non-invasive blood pressure measurement;
- (o)intramuscular and intranasal administration of anti-hypoglycemic agents;
- (p)administration of oral analgesics;
- (q)administration of acetylsalicylic acid

(Schedule 2.2)

- (a)maintenance of intravenous lines without medications or blood products;
- (c)administration of the following oral, sublingual or inhaled medications:
 - (i)anti-anginal;
 - (iii)analgesics;
 - (iv)platelet inhibitors;
- (e)chest auscultation;
- (g)intramuscular administration of epinephrine;
- (h)administration of bronchodilators by inhalation and nebulization.

(Schedule 2.3)

- (b)administration of isotonic crystalloid solutions;
- (c)endotracheal intubation;
- (d)electrocardiogram acquisition;
- (e)initiation and maintenance of non-invasive positive pressure airway devices;
- (f)administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i)anti-emetic — anti-nauseant;
 - (ii)vitamins;
 - (iii)corticosteroids;
 - (iv)opioid analgesics;
 - (v)anti-cholinergic;
- (g)collection of capillary and venous blood samples;
- (h)initiation and maintenance of intraosseous needle cannulation with local anesthetic instillation;
- (i)taking microbiology swabs of dermal and mucosal sites;
- (j>manual defibrillation with cardiac rhythm interpretation;
- (k)electrocardiogram interpretation;
- (l)needle thoracentesis;
- (m)administration of chemical and biological agent countermeasures;
- (n)administration of drug therapy during transportation between facilities
 - (i)in accordance with a treatment plan issued by the patient's attending medical practitioner at the facility from which the patient is being transported, and
 - (ii)after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

**Legal Scope of Practice
Critical Care Paramedic (CCP)**

(Schedule 1.1)

- (a) scene assessment;
- (b) assessment of level of consciousness, skin colour and temperature, pulse, and respiration;
- (c) rapid body survey to identify and attend to any life threatening injuries followed by a secondary assessment consisting of a physical examination, medical and incident history, and vital signs;
- (d) cardiopulmonary resuscitation;
- (g) maintenance of airways and ventilation, including by
 - (i) insertion and maintenance of oropharyngeal airway devices and nasopharyngeal airway devices, and
 - (ii) use of suction devices and bag-valve-mask devices;
- (h) oxygen administration and use of oxygen administration equipment;
- (i) use of automatic and semi-automatic defibrillators;
- (j) wound management not requiring tissue puncture or indentation;
- (k) fracture management and immobilization;
- (l) lifting/loading and extrication/evacuation.

(Schedule 1.2)

- (b) occupational first aid;
- (c) transportation;
- (j) soft tissue injury treatment.

(Schedule 1.3)

- (b) administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i) opioid antagonist;
 - (iii) anti-histaminic;
 - (iv) sympathomimetic agent;
 - (v) procoagulant;
 - (vi) anti-hypoglycemic agent;
 - (vii) anti-pyretic;
 - (viii) non-opioid analgesics;
- (c) insertion and maintenance of airway devices not requiring visualization of the larynx;
- (d) maintenance of intravenous lines, including use of infusion devices and saline locks;
- (e) initiation of peripheral intravenous lines;
- (f) point-of-care testing to inform emergency treatment or transportation decisions or to provide information for subsequent treatment;
- (g) end-tidal carbon dioxide monitoring.

(Schedule 1.4)

- (b) cardioversion and external pacing;
- (f) initiation of external jugular vein cannulation;
- (g) cricothyrotomy;
- (h) gastric intubation and suction;
- (i) maintenance of intravenous lines with medications;
- (j) insertion and maintenance of advanced airway devices which do not require laryngoscopy;
- (l) administration of colloid and non-crystalloid volume expanders;
- (m) administration of the following intravenous, oral, nebulized, endotracheal, intraosseous, intramuscular and rectal medications:
 - (i) anti-arrhythmic;
 - (ii) electrolyte — calcium therapy;
 - (iii) diuretic;
 - (iv) anti-coagulant;
 - (viii) sedative;
 - (ix) anti-emetic — anti-nauseant;
 - (x) histamine antagonist;
 - (xi) anti-convulsant;
 - (xii) alkalizer;
- (n) reduction of joint dislocations involving neurological or vascular compromise;
- (o) administration of drug therapy after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

(Schedule 1.5)

- (b) initiation of arterial lines;
- (c) use of incubators for thermoregulation;
- (d) chest tube insertion;
- (e) initiation of central venous line;
- (f) escharotomy;
- (g) esophageal manometry;
- (h) pericardiocentesis;
- (i) mechanical ventilation management and strategy development.

CCP Endorsements**(Schedule 2.1)**

- (c) spinal motion restriction;
- (e) administration of oral glucose;
- (f) emergency childbirth;
- (h) assistance to a patient with the administration of a medication to the patient, provided that

- (i) if, under the *Pharmacy Operations and Drug Scheduling Act*, a particular medication is available only on the prescription of a health professional, the medication has already been prescribed to the patient by a health professional,
- (ii) the patient has requested the EMA to assist the patient with the administration of the medication,
- (iii) the administration of the medication is related to emergency services the EMA is providing to the patient,
- (iv) the medication is administered as prescribed, and
- (v) the patient is not being transported between health facilities;
- (i) use and interpretation of pulse oximeters and CO-oximeters;
- (j) use and interpretation of glucometers;
- (k) administration of epinephrine by intramuscular auto-injector or intranasal or sublingual preparations;
- (l) topical administration of pro-coagulants and anti-fibrinolytics;
- (m) intramuscular and intranasal administration of opioid antagonists;
- (n) non-invasive blood pressure measurement;
- (o) intramuscular and intranasal administration of anti-hypoglycemic agents;
- (p) administration of oral analgesics;
- (q) administration of acetylsalicylic acid

(Schedule 2.2)

- (a) maintenance of intravenous lines without medications or blood products;
- (c) administration of the following oral, sublingual or inhaled medications:
 - (i) anti-anginal;
 - (iii) analgesics;
 - (iv) platelet inhibitors;
- (e) chest auscultation;
- (g) intramuscular administration of epinephrine;
- (h) administration of bronchodilators by inhalation and nebulization.

(Schedule 2.3)

- (b) administration of isotonic crystalloid solutions;
- (c) endotracheal intubation;
- (d) electrocardiogram acquisition;
- (e) initiation and maintenance of non-invasive positive pressure airway devices;
- (f) administration of the following inhaled, intramuscular, intranasal, intraosseous, intravenous, nebulized, oral, subcutaneous or sublingual medications:
 - (i) anti-emetic — anti-nauseant;
 - (ii) vitamins;
 - (iii) corticosteroids;
 - (iv) opioid analgesics;
 - (v) anti-cholinergic;
- (g) collection of capillary and venous blood samples;

- (h) initiation and maintenance of intraosseous needle cannulation with local anesthetic instillation;
- (i) taking microbiology swabs of dermal and mucosal sites;
- (j) manual defibrillation with cardiac rhythm interpretation;
- (k) electrocardiogram interpretation;
- (l) needle thoracentesis;
- (m) administration of chemical and biological agent countermeasures;
- (n) administration of drug therapy during transportation between facilities
 - (i) in accordance with a treatment plan issued by the patient's attending medical practitioner at the facility from which the patient is being transported, and
 - (ii) after consultation with a medical practitioner who is designated by an employer as a medical oversight advisor and who has advised the EMA to administer the drug therapy.

(Schedule 2.4)

- (a) automated mechanical ventilation;
- (c) urinary catheterization;
- (d) arterial line management and central venous pressure monitoring;
- (e) infusion of blood products;
- (g) collection of arterial blood samples;
- (h) interpreting laboratory and radiologic data;
- (j) chest tube management;
- (k) central line management;
- (l) management of parenteral feeding lines and equipment;
- (m) provision of trans-venous pacing;
- (n) invasive wound management;
- (o) subcutaneous application of a local anaesthetic;
- (p) finger thoracostomy.

Previous Amendments

Date	Version	Description
01.21.26	1.1	
7.2 Procedures		
Wound Management (Invasive)	Wound packing (hemostatic) changed to: 1. Within BCEHS operation scope for PCFP and ITT	
Patient Restraint	Pharmacological restraint changed to: 1. Mandatory consult for PCFP	
8.0 Routes of Medication Administration		
Vascular Routes	Central Venous Line added 1. Mandatory consult for PCFP and ACFP 2. Within BCEHS operational scope for ITT, CCP-MNP, CCP-Adult	
Authorised Medications		
High Risk	Fentanyl changed to: 1. Within BCEHS scope, may require clinical consult for PCFP	
	Hydromorphone changed to: 1. Within BCEHS scope, may require clinical consult for PCFP and ACFP	
	Ketamine changed to: 1. Within BCEHS scope, may require clinical consult for PCFP	
	Lorazepam changed to: 1. Mandatory consult for PCFP 2. Within BCEHS scope, may require clinical consult for ACFP	
	Midazolam changed to: 1. Operational restriction for PCFP	
	Morphine changed to: 1. Within BCEHS scope, may require clinical consult for PCFP	
	Phenobarbital changed to: 1. Operational restriction for PCFP 2. Within BCEHS scope, may require clinical consult for ACFP	
	Enoxaparin changed to: Blue background to reflect CHAMP kit carry	
	Enoxaparin changed to: Blue background to reflect CHAMP kit carry	
Moderate Risk	Levetiracetam changed to: 1. Mandatory consult for ACFP	

02.14.26	2.0	Description
Section 1		
Mission Profiles	Added the Clinical Advisor’s Approach to title.	
	Added approach matrix	
Section 2		
1.0 Response Capability: Transport	Incubator as a transport platform for non-thermoregulation needs added for all levels – This is currently operationally restricted while under review	
Section 3		
Authorised Medications	Clarification of “Red” category updated to reflect the following: Restricted to IFT operational scope and requires mandatory clinical advisor consultation and clearance for use	
	“Operationally Restricted” category removed to reflect potential IFT scope as determined by the Clinical Advisor	
	Clarification of purpose of EMALB Regulation 2.3(n)(i)(ii) with respect to maintenance and management of patients with medications	

03.05.26	2.1	Description
Section 2		
1.0 Response Capability: Transport	Incubator as a transport platform for non-thermoregulation needs added for all levels.	
5.0 Initiating Vascular Access: Arterial Line Insertion	Added 'Restricted for patients less than 12 yrs' in notes section	